15 Coercion – point, perception, process

Dorothy M. Castille[1], Kristina H. Muenzenmaier[2] and Bruce G. Link[3]

[1]*National Institutes of Health, National Institute on Minority Health and Health Disparities, Bethesda, MD, USA*
[2]*Albert Einstein College of Medicine, Bronx, NY, USA*
[3]*Columbia University, New York State Psychiatric Institute, New York, NY, USA*

15.1 Introduction

The use of force in psychiatric treatment continues to be a hotly debated topic. Supporting the coercion-is-necessary perspective is the contention that persons with mental illness fail to behave in socially acceptable ways [1]. At the same time, they may not perceive themselves as behaving in any way outside of acceptable social norms nor find their own behaviour to be a problem to themselves, as it is to others [2,3]. Historically, physicians, legislators and others maintain that the biological condition that underlies some serious mental illnesses results in impaired insight [2,3]. Thus, for many individuals, failure to recognize the need for treatment may require their being forced to enter or remain in treatment.

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Detracting from the coercion-is-necessary perspective are those who argue that the use of force constitutes discrimination and destroys trust in the relationships between mental health consumers and their treatment providers, and may contribute to destabilizing a person's mental health. The more negative consequences of coercive treatment as outlined by consumers are fear, anger and disempowerment, and provoking such labels as 'treatment resistant' and 'treatment noncompliant' [4]. Anger at being forced to take treatment, and fear at being treated against a person's will may trigger people to leave the treatment system or to move to another state where they have not been engaged with the mental health system.

Since deinstitutionalization in the 1970s, a number of strategies have been employed to promote treatment adherence in persons with serious mental illness, particularly those who are seen as treatment resistant. Outpatient commitment is a relatively recent strategy to ensure adherence to treatment in the least restrictive environment. How does the individual with mental illness experience court-ordered assignment to outpatient treatment? Does the order cause people to feel coerced?

Rosenberg and Pearlin [5] assert that coercion represents 'power based on compulsion, sanctions, or the threat of sanctions'. In fact, as Lovell [6] points out, mental health treatment, viewed through the lens of social control, places coercion on a 'pure coercion' to 'bilateral control or voluntarism' continuum. From the perspective of the mental health care professional, the power enlisted to ensure adherence may be persuasive, manipulative, authoritative, contractual or coercive [5]. These approaches represent the effort of one individual to control the behaviour of another; however, all such efforts are not experienced as equally coercive by the recipient [7].

While the concept of coercion is central to a consideration of policies, such as outpatient commitment, measuring it accurately and completely is difficult. Two approaches are prominent and widely used. In the first, one uses the objective fact of an event such as involuntary commitment, exposure to restraints, or assignment to outpatient commitment to gauge exposure to coercion. This approach uses objective conditions experienced by the consumer as the key evidence of coercive treatment and is exemplified in questions such as: what difference does a court order make in patient adherence to treatment? While extremely important and useful, a problem with this approach is that it misses complex processes comprising patient experience and the psychological reaction to those experiences. In the second approach, people are asked questions that reveal whether and to what extent they feel coerced. The exemplar of this latter approach is the Perceived Coercion Scale developed by the MacArthur Research Network on Mental Health and the Law [8–10]. This instrument has been translated into 42 languages [11–13] and applied to hospital admission experiences [14,15] and outpatient commitment [16–19]. Based on its widespread use, we included the MacArthur Perceived Coercion Scale in a study of Kendra's Law, New York State's outpatient commitment law, that compared a sample of people assigned to outpatient commitment and a group of people who were not.

We wondered how strongly the two methods of assessing coercion might be related and what, if anything, might drive feelings of coercion other than the objective fact of court-ordered treatment.

15.2 New York State's Kendra's Law

Our study of coercion in outpatient treatment was undertaken in the context of New York State's Kendra's Law. In 1999, New York became the 42nd of the 50 states to have an outpatient commitment law when the state legislature passed Kendra's Law (New York Mental Hygiene Law 9.60), allowing courts to require 'assisted outpatient treatment' (AOT) for severely ill individuals with a history of noncompliance with treatment and in need of additional support to successfully engage in treatment in an outpatient setting. Accompanying a court order to outpatient treatment may be a bundle of services which is always coordinated by an Intensive Case Manager or an Assertive Community Treatment (ACT) team, often someone with training as a Social Worker, including: clinical services and medication; blood tests or urinalysis to determine compliance with prescribed medications; individual or group therapy, day or partial day programming activities; alcohol or substance abuse treatment and counselling and periodic tests for the presence of alcohol or illegal drugs for persons with a history of alcohol or substance abuse; supervision of living arrangements; and any other service within a local or unified service plan [20]. The law has given priority access to scarce necessities of life, for example housing, clinical outpatient services and assignment to a case manager, to the proportion of the population most severely affected by mental illness and yet most difficult to engage in the treatment system.

In the present chapter we begin with a comparison between the two previously mentioned approaches to measuring coercion. We ask: does a person with a court order perceive him/herself as being more coerced than a person in outpatient treatment without a court order? As we document below, the answer we obtained surprised us, as the group court-ordered to treatment was not significantly higher than the comparison group in perceptions of coercion to outpatient treatment. This anomalous finding led us to ask about the reliability and validity of the Perceived Coercion Scale in our sample, to explore other determinants of perceptions of coercion, and to conduct in-depth qualitative interviews to deepen our understanding about the reasons for our findings.

15.3 Methods

15.3.1 Study sample

Men and women between the ages of 18 and 65 years with a history of serious mental illness, the capacity to give informed consent, and with the ability to speak either

English or Spanish were recruited from various outpatient clinics in two boroughs of New York City (Table 15.1). People court-ordered into assisted outpatient treatment (AOT Group) and people in outpatient treatment (Outpatient Group or OPT) receiving care as usual comprise the study population. Of the 184 people interviewed

Table 15.1 Socio-demographic characteristics of sample population by AOT and OPT status.

	AOT $n = 76$ (41.3%)	OPT $n = 108$ (60.3%)	Total $n = 184$ (100%)
Age – Mean	35.8	37.8	37.0
Sex			
Female	24 (31.6%)	49 (45.4%)	73 (39.7%)
Male	52 (68.4%)	59 (54.6%)	111 (60.3%)
Race			
Black	45 (59.1%)	53 (49.1%)	98 (53.3%)
White	6 (7.9%)	8 (7.1%)	14 (7.6%)
Hispanic	18 (23.7%)	35 (32.4%)	53 (28.8%)
Other	7 (9.2%)	13 (11.1%)	19 (10.3%)
Marital			
Married, common law	5 (6.7%)	12 (11.1%)	17 (9.3%)
Widowed, divorced, separated	12 (16.0%)	25 (23.1%)	37 (20.2%)
Never married	58 (77.3%)	71 (65.7%)	129 (70.5%)
Employment			
Currently unemployed	63 (86.3%)	93 (88.6%)	156 (87.6%)
No. wks unemployed in past year – Mean	46.5 wk	47.3 wk	47.0 wk
Income source[a]			
Public Assistance	10 (13.2%)	13 (12.1%)	23 (12.6%)
SSI (Gold Check)	31 (40.8%)	40 (37.0%)	71 (38.6%)
SSD (Disability)	24 (32.4%)	29 (26.9%)	53 (29.1%)
SSP (Pension) Medicaid	0 (0.0%)	1 (0.9%)	1 (0.5%)
Medicaid	62 (81.6%)	84 (77.8%)	146 (79.3%)
Medicare	20 (26.7%)	21 (19.6%)	41 (22.5%)
Food Stamps	21 (27.6%)	24 (22.4%)	45 (24.6%)
Education			
Less than High School	33 (43.4%)	40 (37.0%)	73 (39.7%)
Completed HS or more	43 (56.6%)	68 (63.0%)	111 (60.3%)
Axis I diagnosis			
Schizophrenia	29 (40.3%)	42 (40.0%)	71 (40.1%)
Schizo-affective disorder	30 (41.7%)	27 (25.7%)	57 (32.2%)
Major depressive disorder	1 (1.4%)	12 (11.4%)	13 (7.3%)
Bipolar disorder	11 (15.3%)	22 (21.0%)	33 (18.6%)
Other	1 (1.4%)	2 (1.9%)	3 (1.7%)

$^{+}P < 0.1$; $^{*}P < 0.05$; $^{**}P < 0.01$; $^{***}P < 0.001$.

[a]Percentages do not sum to 100% because respondents may have more than one source of income and service coverage.

at baseline, 39.7% are female and 60.3% are male. The ethnic distribution of the people in the sample reflects that of the clinics at which respondents were recruited; therefore, there are few respondents in our sample of white (7.6%) or other ethnicities (10.3%), the majority endorsing African descent (53.3%) and Hispanic (28.8%) ethnic identities. The AOT and OPT groups differ very little by mean age: AOT respondents, 35.8 years; and OPT respondents, 37.8 years. Fewer persons on AOT (43, or 56.6%) had completed high school or more years of education than outpatient respondents (68, or 63.0%). Most (70.5%) of the people in the sample had never married. The majority of the respondents in the sample reported being unemployed (87.6%) for an average of 47 of the previous 52 weeks. Most study participants received services reimbursed by Medicaid regardless of AOT or OPT status. While the ethnic distribution of this New York City sample differs from that published by the New York State Office of Mental Health for the entire state, it does reflect the population receiving services in the state on every other dimension and does not differ from published data for New York City recipients (http://bi.omh.state.ny.us/aot/about).

Of respondents, 59.2% reported having been arrested, AOT 50 (66.7%) vs. OPT 59 (54.6%), prior to the baseline interview and 48 (26.1%) of the total study population described a history of more than three arrests. Court-ordered consumers reported significantly more hospitalizations (an average of 9.51 hospitalizations per person) than their non-court-ordered peers (6.26 hospitalizations). These characteristics suggest a group of individuals experiencing more life difficulties than their outpatient peers (Table 15.2).

15.3.2 Measures

The battery of measures administered in this study used multiple-item scales measured at six time points: baseline, 1, 3, 6, 9 and 12 months; however, we report here on baseline results only. For scales containing positively and negatively worded statements, each item is scored so that a high score on the item reflects a high score

Table 15.2 Self-reported clinical and forensic history by AOT and OPT status.

	AOT	OPT	Total
Arrest history			
Ever arrested	50 (66.7%)	59 (54.6%)	109 (59.2%)
Arrested four or more times	24 (31.6%)	24 (22.2%)	48 (26.1%)
Hospitalization			
No. of hospitalizations – Mean	9.51***	6.26	7.88

$^{+}P < 0.1$; $^{*}P < 0.05$; $^{**}P < 0.01$; $^{***}P < 0.001$.

on the construct being measured. For each scale employed, we report the baseline internal consistency reliability (α) and create scale scores by summing the scores for each item in a scale and dividing by the total number of items in that scale when values for at least two-thirds of the scale items are present. This yields scores that locate the scale mean in relation to the response categories for the items, and prevents loss of subjects due to the absence of less than one-third of the scale items. For example, a scale mean of 2 for items using a four-point 'strongly disagree' (0), 'disagree' (1), 'agree' (2) and 'strongly agree' (3) format would tell us that the average respondent is in agreement with the items in the scale.

AOT Assignment Individuals experiencing a court order for mandated treatment within 90 days of the baseline interview constituted the AOT group. Of the 184 study participants, 76 experienced this form of objective coercion while 108 did not.

Perceived Coercion Scale The Perceived Coercion Scale is a five-item version of a scale developed by the MacArthur Research Network on Mental Health and the Law [10,21]. In contrast to the original scale construction that used a 'true/false' response pattern, the scale items were adapted in the present study to ask whether, considering experiences of the past 12 months, the individual 'strongly disagrees = 0', 'disagrees = 1', 'agrees = 2' or 'strongly agrees = 3' with the following statements:

- '...you felt free to do what you wanted about getting outpatient mental health treatment';
- '...you chose to get outpatient mental health treatment';
- '...it was your idea to get mental health treatment';
- '...you had a lot of control over whether you got outpatient mental health treatment'; and
- '...you had more influence than anyone else on whether you got treatment'.

Items were scored such that a high score reflected high perceived coercion.

To evaluate construct validity of the Perceived Coercion Scale, we used a number of measures that, if correlated in the predicted direction, would support the validity of the scale.

Potential correlates to be used as evidence in assessing construct validity include measures of stigma, quality of life, insight, the working alliance, the severity of psychotic symptoms, and the number of involuntary hospitalizations.

Stigma, known to be associated with mental illness, may be hypothesized to correlate significantly with a court order to remain in treatment. We assessed stigma using an eight-item version of Link's [22] perceived devaluation/discrimination measure ($\alpha = 0.74$). The measure asks the extent to which respondents 'strongly

agree', 'agree', 'disagree' or 'strongly disagree' with statements indicating that most people devalue or discriminate against current or former psychiatric consumers.

Quality of Life [23] ($\alpha = 0.91$) is an area in which a reduction in symptoms due to adherence to treatment may bring improvement. Therefore, better quality of life may be negatively associated with the Perceived Coercion Scale. Quality of Life is assessed using 14 items of Lehman's self report scale that asks respondents to rate as 'poor' (1), 'fair' (2), 'good' (3) or 'excellent' (4) multiple domains of life such as living conditions, employment, social life, physical health, level of independence and self-esteem. Items are summed and divided by 14 if values for two-thirds of the items are present, to yield scale scores with a maximum high point of 4 and a minimum low of 1.

The Working Alliance Inventory ($\alpha = 0.91$) [24], expected to be negatively associated with high perceived coercion because coercion is hypothesized to reduce trust between the individual and the case manager, is assessed using an eight-item measure. Respondents are asked how often ('very often', 'fairly often', 'sometimes', 'almost never' or 'never') their case manager does things for them or makes them feel a particular way. Example items are 'How often are you confident in your case manager's ability to help you?' and 'How often do you feel that your case manager appreciates you?'

Insight expected to be impaired by symptom severity may be expected to improve with treatment and thus to be associated with low perceived coercion. The Insight measure comprised a four-item scale based on three slightly modified items of the ITAQ (the Insight and Treatment Attitudes Questionnaire [25]) and one item of the ROMI (Rating of Medication Influences Scale [26]) ($\alpha = 0.73$), including these questions: 'Do you think you have mental illness now?', 'Have you ever had mental illness?', 'Have you ever needed treatment for mental illness?' and 'Do you believe that you don't have a mental illness?'

Severity of psychotic symptoms occurring in the past three months was assessed using both a fixed-format, self-report symptom scale and a scale of symptoms derived from the clinician-administered Structured Clinical Interview for Diagnosis (SCID). The fixed-format scale ($\alpha = 0.95$) is a 10-item scale designed especially for this research and targets so-called threat/control override symptoms (TCO) that have been proposed as symptoms that might lead to violence [27]. We used the clinician-administered SCID to construct a scale of psychotic symptoms ($\alpha = 0.62$) by summing ratings of the presence (1) or absence (0) of delusions of reference, persecutory delusions, delusions of control, thought broadcasting and grandiose delusions.

The number of involuntary hospitalizations is assessed by asking at baseline how many times the study participant had been hospitalized against their will. Because the resulting frequency distribution is skewed, we add one to the number of involuntary commitments and then take the natural log of the sum to create a metric that varies from 0 to 2.77.

15.3.3 Qualitative interviews

Open-ended interviews were conducted in conjunction with the nine-month interview with a sample of 20 respondents, 9 persons on AOT and 11 Outpatient (OPT) group members. Interviewer recommendations influenced the choice of persons to be interviewed based on the respondent's ability to speak about his or her experience and to represent as wide a range of perspectives on the experience of coercion as possible. We wanted the qualitative subsample to include both individuals who felt coerced and individuals who did not feel coerced, in both the AOT and the non-AOT groups. Interviews were audio taped, transcribed and coded thematically by two coders (the first author and a social worker with more than 30 years of experience and trained in thematic coding). Disagreements in coding were discussed until agreement was reached. The software ATLAS.ti was used to assist in the analyses of the coded transcripts and to permit some quantitative analyses of qualitative data.

15.3.4 Recruitment

Recruitment for the overall project proceeded when the treating psychiatrist, psychologist or social worker approached the patient about study participation. If the patient agreed to participate, a project research interviewer contacted him/her, introduced the study and extended a formal invitation to the individual to participate. No one who was incapable of giving informed consent was referred by the health care provider to project interviewers. Participants were given a small monetary compensation at the completion of the interview [28,29].

15.3.5 Statistical analyses

Data were analysed in the software package SPSS 14.0 [30] using basic descriptive statistics and linear regression models. Statistical significance at $P < 0.05$ or higher is indicated where appropriate.

15.4 Human subjects protections

The project has Institutional Review Board approval at the New York State Psychiatric Institute, Bronx Psychiatric Center, Creedmoor Psychiatric Center, Bronx-Lebanon Medical Center and the New York State Office of Mental Health. Further, all participants are covered by a Certificate of Confidentiality issued by the Federal Government Services that allowed us to treat the information obtained as privileged in any legal proceeding.

15.5 Results

15.5.1 Perceived coercion in AOT and OPT

We compared AOT and OPT respondent means on the Perceived Coercion Scale using two methods of calculating scores for that scale. The first method, 'non-weighted', recoded the items for a high score to represent high perceived coercion and took the mean with values for, at least, three of the five scale items. The second method, 'weighted', used a 'true–false' fixed response, recoding 'strongly disagree/disagree' responses as 0 and 'agree/strongly agree' responses as 1, and weighting the characteristics of coercion according to the salience of each item to the concept of coercion as identified by Gardner and colleagues [10]. As Figure 15.1 shows, there are no statistically significant differences in the means for AOT versus OPT for either the weighted or non-weighted calculation of the Perceived Coercion Scale. In the remaining analyses reported on here, we use the un-weighted calculation of the MacArthur measure of perceived coercion and refer to it as the Perceived Coercion Scale (PCS).

Using this scale we observe non-significantly higher perception of coercion in the AOT group (mean = 1.44) as compared with the OPT (mean = 1.33). These results differ from other comparisons of perceived coercion in groups that differ in objective coercion such as at hospital admission [15,31], in mental health courts [32], and in outpatient commitment [32,33]. In each of these instances there is a significant difference in perceived coercion between the objectively coerced group and the

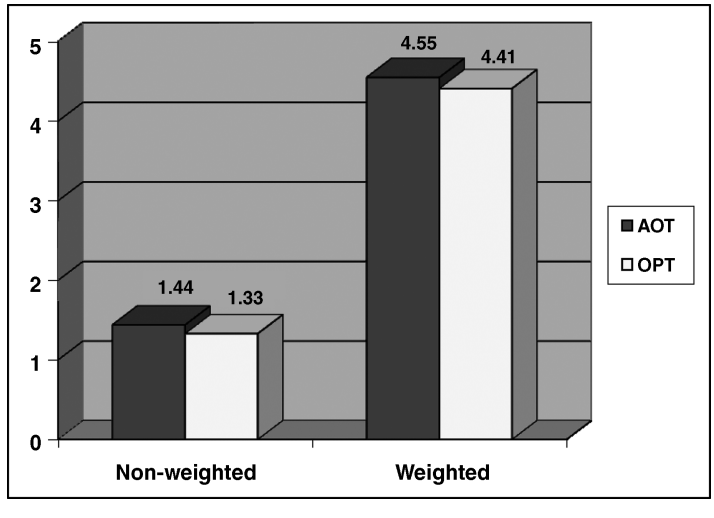

Figure 15.1 Comparison of AOT and OPT mean scores using two calculations of perceived coercion.

comparison group, while in the current study there is no significant difference between the groups.

15.5.2 Perceived coercion and AOT dose

Finding that the two groups of consumers perceived themselves as being relatively equally coerced led us to a number of further questions. Perhaps, persons with different levels of access to case management services might experience coercion differently, a discrimination obscured by the dichotomous grouping of subjects into either AOT or OPT. To examine this possibility, we compared Perceived Coercion Scale means for persons who had no case manager, persons with a case manager but no court order, and persons on court order for three months or less, with persons who had more than three months on an AOT court order. We found that those groups of individuals exposed to AOT for longer periods of time had slightly higher means on the Perceived Coercion Scale (no OPT with no case manager, mean = 1.31 ($n = 44$); OPT with case manager, mean = 1.34 ($n = 64$); AOT court order 90 days or less, mean = 1.41 ($n = 26$); AOT court order more than 90 days, mean = 1.46 ($n = 49$)). However, there were no significant differences in the means across the groups.

Further, when we compared PCS scores for persons on AOT with those not on AOT in multiple regression analyses (not shown), controlled for age at baseline, sex, race, marital status and education, the association between AOT status and perceived coercion remained non-significant.

One potential explanation for the lack of a difference between committed and not committed groups on perceived coercion could be measurement problems associated with our use of the PCS. As a result, we assess whether the PCS performs reliably and shows evidence for validity in this New York sample of people with serious mental illness. The internal consistency reliability of the PCS assessed via Cronbach's Alpha is 0.86, which reveals a high level of reliability, suggesting that the unreliability of the measure is probably not the source of the no-difference finding.

To assess construct validity, we determined whether the PCS scale correlated in this context as predicted with measures of other constructs. Again, using the concerns of consumer and human rights advocates, we expected that perception of coercion would significantly, negatively influence the working alliance between the person and the case manager. On the other hand, the competing hypothesis suggested by family members and mental health care professionals led us to expect that indicators of illness severity (lack of insight, number of prior hospitalizations and delusions of harm) should be strongly associated with higher perception of coercion. We recognize that additional factors that may have a bearing on perceived coercion, for example the stigma of mental illness and of the court order itself, may contribute to a

Table 15.3 Construct validity of the perceived coercion scale

	Perceived coercion Pearson Correlation
AOT ($N = 183$)	0.086
Working Alliance Inventory ($N = 133$)	−0.339**
Insight ($N = 181$)	−0.236**
Harm Delusions ($N = 183$)	0.126 +
Prior Involuntary Hospitalizations ($N = 179$)	0.331**
Stigma Devaluation-Discrimination Scale ($N = 178$)	0.260**
Stigma: AOT Scale ($N = 71$)	0.378**
Quality of Life ($N = 182$)	−0.223**

+ $P < 0.1$; * $P < 0.05$; ** $P < 0.01$; *** $P < 0.001$.

person's feeling more coerced. A positive consequence of mandated treatment may be an improvement in the individual's quality of life. Thus, we test, as well, the association of stigma and quality of life with perceived coercion.

We found:

- The event of a court order to AOT was not significantly correlated with high perceived coercion (Pearson Correlation Coefficient = 0.087; see Table 15.3)
- Coercion negatively affects the patient's relationship with the case manager.
- Perception of coercion is associated with markers of illness severity (low insight, number of prior hospitalizations and delusions of harm) and feelings of being stigmatized.
- The higher a person's perception of coercion, the lower the self-reported quality of life.
- We also examined and found no significant correlation of our measure of perceived coercion with the key demographic variables of age, sex, race/ethnicity, years of education and marital status (see Table 15.4).

In sum, the Perceived Coercion Scale, whether constructed using a weighting scheme or a simple mean with at least three of five items present, is reliable and shows evidence for construct validity in the study population.

Table 15.4 Comparison of AOT and OPT groups on perceived coercion by sex, age, race, marital status and education

Coercion	AOT/OPT	Sex	Age	Race	Marital	Education
Correlation coefficient	0.087	0.045	−0.026	0.038	0.021	−0.014
Significance	0.249	0.541	0.723	0.609	0.783	0.850
Ns	183	183	182	183	182	183

15.5.3 Perceived coercion and the use of leverage in outpatient commitment

Having increased our confidence that the finding of no difference between AOT and OPT groups on perceived coercion was not an artefact of poor measurement spurs us to further consideration of our finding. For example, could it be that some participants did not realize they had been court-ordered into treatment due to the direct effects of the disease process, and thus did not see themselves as coerced? Did the manner in which the judge conducted the AOT hearing or the case manager discussed treatment options effect the patient's feeling of being coerced? Is the use of force in treatment so pervasive that a court order adds nothing noticeable to the individual?

Monahan and colleagues contend that the court order is only one form of leverage used to ensure adherence to treatment [7,34–37]. Included in the battery of measures administered to study respondents were items developed by the MacArthur Violence Risk Network to measure aspects of leverage that might be used with persons with serious mental illness. 'Did you ever live somewhere where you felt you were required to stay in mental health or substance abuse treatment (or take your medication)?' assessed the use of housing as leverage. Other questions evaluated leverage of outpatient commitment and reception of benefits through a representative payee. Of the 75 individuals on AOT asked whether they felt 'required to stay in mental health treatment' to obtain housing, 56% ($n = 42$) of the respondents said they did *not* feel required to stay in mental health treatment as compared with 68.2% ($n = 73$) of the OPT respondents, significant at the $P < 0.064$ level. At the same time, some of the OPT group, 31.8% ($n = 34$), felt obligated to remain in treatment without the presence of a court order. Further, when housing, representative payee and outpatient commitment forms of leverage were summed and a comparison of means performed, there was no significant difference between persons on AOT and OPT in this sample. Nor was there a correlation between cumulative forms of mandated treatment and either court-ordered status or perception of coercion. Either an important dimension of the experience of the court order is not being tapped by the Perceived Coercion Scale, or the scale itself does not touch significant mediating aspects of coercive treatment that negatively affect treatment adherence.

15.5.4 Qualitative results: why is there no difference?

What does explain this lack of expected differences? To answer this question, we drew on qualitative interviews, data from participant observation and, at times, survey results, as they proved relevant to the qualitative data. Open-ended interviews were completed with a subsample of 20 men and women in AOT (9) and not in AOT (11) interviewed at the nine-month follow-up interview. All individuals approached

agreed to talk about their responses with Dr Castille as part of the follow-up interview. Those discussions were transcribed and the content coded following conventions described by Miles and Huberman [38], Ryan and Bernard [39] and others, and analysed specifically to build an understanding of what does and does not make a person feel coerced. There were no significant differences between the subsample interviewed in-depth about perception of coercion and the remaining un-interviewed sample on any demographic variable. Participant observation data came from the involvement of one of us (Dorothy Castille) in conducting the quantitative study and thereby spending time in the clinical settings. Notes from these observations were recorded and reviewed for relevance to the issue of coercion. For each person quoted, we give their sex, age, group membership and their perceived coercion score as compared with the mean for their group.

We organized our results in the form of findings from our qualitative study concerning key contextual drivers of the experience and perception of coercion. Specifically, we identify three such reasons including: (i) the ubiquity of coercion; (ii) compliance over confrontation; and (iii) a valued services perspective that addresses the availability of an Intensive Case Manager, housing, employment, and the ability to achieve goals. We treat each theme in turn and support our discussion with excerpts from participant narratives.

15.5.5 The ubiquity of coercion

Does coercive experience form such a consistent backdrop to life that treatment feels equally coercive to those with and without a court order? A number of explanations for this are possible. All respondents in the study had considerable histories in the mental health system. As previously mentioned, we found number of prior involuntary hospitalizations to be significantly, positively correlated with high perceived coercion (0.331^{**}, $P < 0.01$), lending some support to this hypothesis. Additionally, having been brought to the emergency room in handcuffs by law enforcement personnel, having been subjected to restraints and/or seclusion, or having been medicated over objection, comprise significant coercion. Moreover, coercive practices take place on a daily basis throughout a hospitalization where rules narrowly constrain behaviour. The in-hospital treatment regime requires participation in scheduled activities. Certain spaces, such as a person's room, can only be used at certain times and usually under circumstances that are not private. Medication is administered at regular supervised intervals; refusal of medication may be met with court orders to medicate over the patient's objection, and so forth. For example, 'As far as taking my meds, I been taking medication for the longest time, ... they watch you take your meds so I couldn't not take them over there because that would get written up...' (*38-year-old, male, AOT, CO*[1]*mean = 0.80 vs. AOT CO mean = 1.44*). Coercive practices may extend to residential treatment

centres and supervised housing. Thus coercion experienced daily may make the court order one more in the sequence of coercive procedures [40].

15.5.6 Compliance over confrontation

For some individuals, the threatening aspect of AOT carries weight in two ways: as a heavy, punitive hand – 'AOT, it's like you go or you're in trouble' or 'it were just threatening if I had missed a day or something like that, they would, um, hospitalize me again' (*30-year-old, male, AOT, CO mean = 1.00 vs. AOT CO mean = 1.44*) – and as a forced choice – 'I agreed to comply. . . They would have kept me. It would have been a big thing. So I just agreed' (*29-year-old, female, AOT, CO mean = 1.80 vs. AOT CO mean = 1.44*). Here, going along with the court order represents the path of least resistance for discharge, knowing when protest is more trouble than it is worth, and 'choosing' compliance over confrontation.

15.5.7 A valued services perspective – benevolent manipulation

Upon discharge, coordination of essential services including clinical services and medication management, housing, money and legal advocacy are delivered by different agencies. For example, medication and clinical services may be funded through Medicaid and Medicare; housing, provided and administered through residential services agencies; money, supplied through Supplemental Security Income; and legal advocacy, made available through organizations such as MFY Legal Services [35]. The case manager or an Assertive Community Treatment (ACT) Team, assigned via the mechanism of a court order, coordinate services that may be impossible for the patient to organize and access without that support. In addition to bridging the hospital–community gap and coordinating services located in different places, for some the treatment structure provided for the person on AOT creates a context that reduces some of the complex and confusing pressures experienced by the individual transitioning to community life: '. . .it [AOT] took a weight off of my shoulders' (*38-year-old, male, AOT, CO mean = 0.80 vs. AOT CO mean = 1.44*). Put another way,

R: Well, AOT is something like, it put you in the right path. They have a lot to offer because if you do what you've been told and you're clean and sober, they would help you find an apartment. They will help you buy furnitures. They make sure that you get your benefits. They make sure you get your medication. . . Make sure that you are, you are responsible for yourself. You have access to have your apartment. And you know, it's pretty helpful. . . It doesn't bother me. (*39-year-old, male, AOT, CO mean = 1.40 vs. AOT CO mean = 1.44*)

The coordination of services (housing, clinical outpatient services and case management services) being offered with AOT is highly valued by consumers. Services mentioned by participants as being particularly important and influential, for example case management and housing, are discussed below along with two areas that may be facilitated by services offered through AOT: employment and attainment of goals.

Treatment alliance and intensive case management

Intensive case management bridges inpatient and outpatient services, hospital life and life in the community. Case managers see consumers at least once weekly and are responsible for assisting the patient with clinic appointments, the coordination of vital financial support, locating appropriate housing, reviewing and applying for treatment options that provide vocational training, securing a job, and easing the transition from hospital to community. Study respondents cited case managers as one of the important reasons for feeling that the 'program did wonders' for them (*38-year-old, male, AOT, CO mean = 0.80 vs. AOT CO mean = 1.44*). For example, a case manager advocate helps a client to locate specific service and training programmes: 'Some programs, you know, don't live up to their expectations' (*36-year-old, male, AOT, CO mean = 1.20 vs. AOT mean = 1.44*). Help with negotiating the system features prominently in reduced feelings of coercion. Where a case manager can be empathic, able to hear the client's concerns and goals, and facilitate attaining those ends, the individual does perceive the interaction as empowering rather than as coercive [41].

On the other hand, the presence of a case manager who uses coercion or manipulation that is not benevolent to secure compliance with treatment may result in patient anger and frustration and the feeling that the programme is 'too strict'. These emotions may not be reflected in the person's mean score (for example, 1.40 as compared with the AOT group mean of 1.44) on the Perceived Coercion Scale because experiences may not come to mind in association with survey items, but emerge only in recounting events: 'She talk nice to everybody, but when it comes to me, she... give me a hard time... When she talk to my brother, she tell him all my personal business...' (*39-year-old, male, AOT, CO mean = 1.40 vs. AOT CO mean = 1.44*). For this respondent, when the case manager turned to family members to gain greater control over her client, the intervention injured and angered the respondent, making him less likely to comply with treatment and leaving him feeling more coerced. Breaking confidentiality, even in the service of gaining treatment adherence, may destroy rather than enhance compliance. This supports the contention that absence of perceived fairness and apparent lack of procedural justice are important components of the experience of coercion [7]. Criticism of AOT may be expressed in terms of the difficult role of the case manager, but the services

represented by being assigned to a case manager are only part of the picture. While the case manager–client match on race, sex and age can contribute to strengthening the bond between case manager and client, empathy and respect between the case manager and client appear to be the key to whether a client feels empowered or coerced.

Special efforts to engage with the patient, provide education about treatment, medication and available resources, and include the patient in treatment planning can improve quality of life, and empower the patient with a sense of choice and control. These forms of more benign coercion can be experienced as helpful and supportive [6], and some of the participants were able to articulate clearly the importance of a trusting relationship with the case manager, despite the coercive practices embedded in a psychiatric hospitalization experience.

I: And how did... you come to be on AOT?
R: Because when I was in... Hospital, I think for the second or third time, the doctor there suggested that AOT would be beneficial to me, help me, because I have a violent history and they wanted me to be able to have someone to talk to on the outside, so they took me to court and I agreed to comply, because I was non-compliant with medicine and I wasn't going to my program... They would have kept me. It would have been a big thing. So I just agreed... and then when I met her [case manager]... She came to visit me in the hospital a couple of times... So by the time I agreed to, I agreed to it because me and her already established a good relationship. (*29-year-old, female, AOT, CO mean = 1.80 vs. AOT CO mean = 1.44*)

One outpatient respondent with a case manager but not court-ordered into treatment highlights just how significant the help of a case manager can be: 'Like, right, now I need her more than ever because I'm getting ready to move from here, so she's going to help me find housing and stuff like that' (*18-year-old, male, OPT, CO mean = 1.60 vs. OPT mean = 1.33*).

Housing

Housing represents a valued resource to participants in this study that can be a source of coercion or an aid to community re-entry. For example, Monahan and colleagues identified outpatient commitment as one of four forms of leverage used to secure an individual's adherence with treatment requirements along with housing, money and criminal justice [35]. In New York State, an AOT order moves the individual to priority consideration for housing. As one young man who did not perceive himself as being coerced put it: '... in order for me to go to

the B___ [a supervised residence on the hospital campus], they had to put me on AOT... that was the only way I could get to B___.' In this case, and that of several other men and women in this study, the court order was the pathway to secure housing, a means to force the system to provide scarce resources. What statistically amounts to multiple forms of leverage may not be perceived as coercive when the patient feels his/her needs are being met – evidence of a more nuanced reality.

Employment

Employment is a marker of adulthood, a goal toward which men and women strive, and a vehicle for recovery and community reintegration. Work emerged as a frequently mentioned theme in both AOT and OPT interviews. Some study participants cited rigidity of the system set in place through a court order as one reason for feeling coerced by being on AOT. Where the demands of prescribed treatment, attending programmes, receiving medication interfere with the ability to find and keep a job, self-esteem is undermined and the path to recovery obstructed. A 52-year-old male on AOT (*CO mean = 1.20 vs. AOT mean = 1.44*) expressed it this way: 'Well, in a way they teach you how to have motivation, to be responsible, but in another way they don't look at your side, like for example, me, I don't have a lot of clothes so I would like to work to buy me new clothes. But they don't want me to work. That's the bad thing about AOT, you know' (*52-year-old, male, AOT, CO mean = 1.20 vs. AOT CO mean = 1.44*). A 44-year-old, male outpatient also spoke of work and treatment, 'Right now all I need is a job, a possible (*sic*) go to church, and a job, and live a normal life and they don't, they say, "Well, you're taking medication, you're doing much better, don't go too fast, go slow"' (*44-year-old, male, OPT, CO mean = 2.20 vs. OPT CO mean = 1.33*). Work and school form important connections to life in the community and 'normality':

> 'Cause I don't like being inpatient. I like having freedom. I like being free and I'm working towards having my complete freedom and returning back to society and to work. That's why I'm involved in school right now. (*29-year-old, male, AOT, CO mean = 0.80 vs. AOT CO mean = 1.44*)

Work though is a double edged sword, 'I would say that making more than $200 a week would make me lose my medical benefits' (*28-year-old, male, OPT, CO mean = 1.60 vs. OPT CO mean = 1.33*). Consumers both on and not on AOT balance recovery-orientated goals and desires with the reality of securing and maintaining stable housing, working enough to advance recovery but not too much to cause a loss in benefits, and attending treatment to retain the useful and supportive features of the mental health care system.

Attainment of goals

The final theme emerging from participant narratives was the importance of having and attaining goals. The ability to develop and work toward achieving goals challenges the recipient of treatment for serious mental illness. Side effects of psychotropic medication include such overwhelming fatigue and lethargy that maintaining a future orientation is a difficult though important task.

> There's [*sic*] a lot of people in here who's not doing well...They just get their medication and come here and go to sleep or go back and go to sleep. They not aiming towards anything. I'm aiming towards getting my own apartment, you know, eventually having kids and things like that. You know, I'm aiming towards the future... (*18-year-old, male, OPT, CO mean = 1.60 vs. OPT CO mean = 1.33*)

15.6 Discussion

Validation of the Perceived Coercion Scale in a New York sample of persons with serious mental illness, court-ordered and non-court-ordered to remain in outpatient treatment is the primary goal of this study. We find that the Perceived Coercion Scale is a psychometrically sound instrument with high internal consistency reliability, and some evidence of construct validity. We expected to find the Perceived Coercion Scale to be significantly associated with the event of the court order, because the authoritative and official force exerted a strong impression on the patient through appearing before a judge and receiving an order from the court to remain in treatment following discharge from the hospital. We found that the scale is significantly associated with poor alliance with the case manager at baseline, poor insight into illness, poor quality of life, and endorsement of more psychotic symptoms, as predicted. It is also significantly associated with a history of a higher number of prior involuntary hospitalizations. These findings lend some support to conflicting hypotheses about the positive and negative role of the use of force in psychiatric treatment. However, in this sample, the Perceived Coercion Scale does not differentiate statistically between persons court-ordered into outpatient treatment and those not court-ordered, failing to replicate findings of other coercion researchers [14–17,42–44]. Interpretation of these complex and somewhat conflicting findings led us to use qualitative methods to deepen our understanding of the concept and process of coercion in the experiences of the men and women in the study.

Qualitative interviews with men and women court-ordered into outpatient treatment and not court-ordered suggested that perception of coercion is highly context dependent. While perceived coercion may be easily measured by a survey instrument, the experience may be more nuanced, and statistical results may not be

easily interpreted. For example, the decision to accept or comply with a court order may be a 'forced choice' – agree and be discharged from the hospital or resist and remain hospitalized or, worse yet, overtly forced to comply. For the individual aware of personal needs and available options, a court order may be part of a strategy to obtain scarce and highly desirable services such as a case manager and housing, thus may not be experienced as coercive. Many people with serious and persistent mental illness have no permanent housing, in part due to their inability to manage their lives – including a job, money and relationships. The availability of supervised housing is limited, and applicants are subject to acceptance by residential providers. AOT recipients gain priority consideration for housing, a condition for hospital discharge. A court order shortens the wait for an available slot once the person has been accepted. From this perspective, the law encumbers the system to provide services to recipients. So there is potential in the system itself to assist the person needing services, changing the leverage implicit in the use of outpatient commitment and housing to secure treatment adherence into focused problem solving [35].

Case management services and the manner in which they are delivered, more than the court order, seem to influence perception of coercion. The degree of fit between the case manager and the person receiving services is very important. Specific qualities such as the ability to hear the needs of the patient and to help to meet those needs, to use personal commonalities of sex, age and race to establish or strengthen an alliance with the client, and to use persuasion, can transform a potentially demeaning and stigmatizing relationship into one of support and empowerment [41]. Broken trust between case manager and client leads the client to feelings of anger, frustration and disgust with both the case manager and the mental health system.

A culture of coercion, as has long existed in the lives of persons treated for serious mental illness, means that events, attitudes and behaviours, ways of being treated by others, comprise so much of the unspoken texture of life that it is only noticeable in contrast to a culture where coercion is not as pervasive. Men and women with numerous prior psychiatric hospitalizations, including involuntary commitment, became part of that culture long ago, starting with their early treatment, and, by the time of our study, had experienced so much coercion that it had become difficult to differentiate all coercive events in the surrounding social environment. Freedom, for them, is constructed only within the context of a culture of coercion.

Recognizing the limited good that coercive treatment practices impart, interventions can be designed to diminish the need to use coercion. In our qualitative interviews, the consumers for whom outpatient commitment appeared to work the best were those who received services from people who treated them as equal and important, deserving of respect, and as a valued member of the community; that is, where the power employed was persuasive [6] and decisions were made collaboratively.

15.6.1 Limitations

The findings reported on here are limited by size of the overall study sample and the sampling methods necessitated by the inability to randomly assign people to different standards of care. Outpatient and Assisted Outpatient Treatment project participants differed little on any of the demographic characteristics collected, and the demographic characteristics of the qualitative subsample do not differ from those of the larger sample. Neither do the sample characteristics reported on here differ significantly from data collected by the New York State Office of Mental Health at the state level, on a much larger group of consumers court-ordered into treatment.

15.7 Conclusion

The experience of coercion is more than can be measured by either a point or isolated coercive incident or through a measure using survey-type items. Coercion, rather, refers to a process that is sometimes de-emphasized in the individual's narrative about their experience, sometimes recognized for what it is, and sometimes alienates the coerced individual from other influences in their environment. The scope of the long-debated value of coercive treatment must be re-examined with a broader view of individual history and context included in the formulation. Moreover, ready access to high-quality treatment services is recognized and valued highly by consumers, suggesting that treatment emphasis be targeted toward more and better services for all consumers. This study identifies coercive treatment as a point ripe for intervention and rethinking.

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Endnote

[1] CO is the coercion measure, designating the mean of the respondent as compared with the mean for the group.